Atezolizumab 1680 mg · every 4 weeks · lung cancer

Lung cancer treatment settings Lung settings: previously treated NSCLC; first-line PD-L1-high metastatic NSCLC; adjuvant NSCLC after surgery and platinum chemotherapy; ES-SCLC maintenance after induction. Urothelial uses in the shared clinical protocol require their own patient discussion.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Atezolizumab 1680 mg every 4 weeks

Active medicines: atezolizumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis or premedication is usually required; individual rescue prescription as indicated.
- Carry immune alert card and report new symptoms immediately.
- FBC, renal/liver profile and glucose each cycle; thyroid every 3–6 weeks.
- Effective contraception during treatment and at least 5 months after the last dose; discuss breastfeeding.
- Current NCIS v8 medical-oncology guidance reviewed. Its low/moderate/high cytotoxic default prescriptions are not applied to minimal-risk atezolizumab monotherapy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00593 v10a · pNCCP 00593 pp7–8 · NCCP 00593 v10a supportive care

Sources checked 2026-10-29 · SACTCheck v0.84.26 · Regimen-level drafts; prescriber and oncology-pharmacy verification required